Overcrowding and Surge Capacity Plan

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1. Purpose

This is the approved procedure for handling hospital and Emergency Department overcrowding at CareOps General Hospital. It defines three capacity levels, the indicators that trigger them, the actions to take, and the rules for stepping down. Thresholds are synthetic internal policy. Bed occupancy above about 85 percent is a commonly cited point where hospitals start to have difficulty admitting patients, delays increase and safety risk can rise, which is why the plan begins at 85 percent.

2. How a level is declared

A level is triggered when any one of the listed indicators reaches the threshold. If different indicators point to different levels, the highest level applies. The Duty Manager confirms the level and records the time. The dashboard shows the level as an alert and the AI assistant should quote the triggering indicator when explaining it.

3. Level 1 Alert

Level 1 applies when hospital-wide occupancy is 85 to 89 percent, or ED occupancy is 80 to 89 percent, or ED average waiting time is 30 to 44 minutes, or more than 5 admitted patients are boarding in the ED. Actions are as follows. The Charge Nurse of each ward lists patients who can be discharged today and the Bed Manager is informed. Housekeeping prioritises cleaning of vacated beds so that the turnaround is within 30 minutes. The ED opens the fast track area. The Bed Manager holds an extra bed huddle. Laboratory and radiology are told to prioritise ED samples and scans.

4. Level 2 High

Level 2 applies when hospital-wide occupancy is 90 to 94 percent, or ED occupancy is 90 to 94 percent, or ED average waiting time is 45 to 59 minutes, or 6 to 10 admitted patients are boarding in the ED, or staff availability is Orange. Actions are all Level 1 actions plus the following. The Duty Manager takes charge and informs the Medical Superintendent. The discharge lounge is opened so that discharged patients wait there and free their beds. Consultants review all patients expected to be discharged within 24 hours before 10:00. Boarded patients are moved to inpatient wards, including approved hallway beds in wards, as soon as a bed is assigned. Off-duty nurses and float pool nurses are called in under the call-back procedure. Non-urgent elective admissions are reviewed for postponement. Up to 20 surge beds in the Day Care and Recovery area may be opened with the approval of the Duty Manager. Nearby departments are asked to lend equipment and staff.

5. Level 3 Critical, full capacity protocol

Level 3 applies when hospital-wide occupancy is 95 percent or more, or ED occupancy is 95 percent or more, or ED average waiting time is 60 minutes or more, or more than 10 admitted patients are boarding in the ED, or staff availability is Red. Level 3 can only be declared by the Medical Superintendent. Actions are all Level 2 actions plus the following. All non-urgent elective surgeries and admissions are postponed. All 40 surge beds may be opened. The

emergency operating theatre stays free and unused theatres may be used as recovery space. The Medical Superintendent informs the Hospital Administrator. Ambulance services are asked to take non-critical patients to other hospitals, while critical patients are always accepted. A command meeting is held every 2 hours. Mutual aid is requested from neighbouring hospitals for beds and staff. Additional staff may be asked to work extra shifts within the overtime limits of the Staffing Policy.

6. Example scenario

Suppose the ED has 150 arrivals in a few hours, average waiting time is 42 minutes, ED occupancy is 91 percent, and staff availability is low. Waiting time of 42 minutes falls in Level 1. ED occupancy of 91 percent falls in Level 2. Low staff availability falls in Level 2. The highest level is Level 2, so the Level 2 actions apply. The recommended response is to inform the Medical Superintendent, open the discharge lounge, expedite discharges, move boarded patients to wards, call in float pool staff, and check beds and equipment in nearby departments. If waiting time continues to rise to 60 minutes, the plan escalates to Level 3.

7. Stepping down

A level can be reduced when all triggering indicators have stayed below the threshold of that level for 2 consecutive hours. The step down is done one level at a time. The Duty Manager records the time and informs the departments. Surge beds are closed and patients are moved to licensed beds only when it is safe.

8. Review after an event

Within 3 working days after any Level 2 or Level 3 event the Quality Office prepares a review that lists the cause, the duration, the number of patients affected, the actions that worked and recommended changes. Recurring causes such as weekend discharge delays or late laboratory reporting are added to the improvement plan.